Wound Care – First Assessment Visit Note

Patient Information

Field	Entry
Patient Name	John Smith
MRN	145892
DOB	02/18/1959
Date of Service	09/23/2025
Provider	Bill Under, DPM
Location	Wound Care Center – Illinois

Reason for Visit / Chief Complaint

Chief Complaint	Details
Reason for visit	Patient presents for evaluation of multiple chronic non-healing wounds: diabetic foot ulcers and venous leg ulcer.

History of Present Illness (HPI)

Parameter	Details
Onset of wound	Multiple wounds noted over last 8–12 weeks
Etiology	Diabetic foot ulcers (neuropathic/pressure), venous ulcer
Duration	2–3 months
Previous treatments attempted	Daily dressings at home, topical antibiotics, compression stockings, oral antibiotics x2 weeks
Current dressing regimen	Gauze with saline, changed daily
Pain	6/10 burning pain at L plantar foot ulcer; 4/10 intermittent throbbing pain at venous ulcer
Functional impact	Difficulty ambulating, unable to stand >30 min, limited ADLs
Compliance with care	Fair; reports inconsistent dressing changes and compression use

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Past Medical History

Comorbidity	Present (Y/N)	Notes
Diabetes Mellitus (HbA1c)	Y	Last HbA1c: 8.4%
Peripheral Vascular Disease	Y	Mild PAD
Venous Insufficiency	Y	Documented on Doppler
Neuropathy	Y	Bilateral lower extremities
Chronic Kidney Disease	N	
Smoking History	Y	1 pack/day, 30 years
Other	HTN, Hyperlipidemia	Controlled with meds

Review of Systems

Symptom	Present (Y/N)	Notes
Fever/Chills	N	Denies
Malaise	Y	Reports fatigue
Pain at wound site	Y	Variable, see HPI
Drainage/Odor	Y	Moderate drainage from plantar foot wound, mild odor
Other systemic complaints	N	None reported

Physical Exam

Parameter	Findings
General	Alert, oriented, mild discomfort
Vitals	BP 146/88, HR 92, Temp 99.1°F, SpO_2 95%
Extremities	Edema present bilaterally (2+), diminished pulses DP/PT, decreased sensation in both feet, limited mobility

Vascular Assessment Findings:

- ABI: R = 0.82, L = 0.76
- Doppler: biphasic signals, mild arterial insufficiency
- Pedal pulses diminished bilaterally

Complete At Risk Foot Exam

- **Class Findings:**

1. **Class B: (1**) decreased hair growth, nail thickening, and discoloration
2. **Class C: (2**) edema and cold feet

The patient meets Class Findings Criteria by having at least 1 Class B Finding and 2 Class C Findings.

- **Gait:** standard walker. limited household ambulator.

- **Right Shoe Size:** 12.0

1. **Width:** Wide
2. **Measuring Device:** Brannock Device

- **Left Shoe Size:** 12.0

1. **Width:** Wide
2. **Measuring Device:** Brannock Device

- **At Risk Foot Exam**
- **Peripheral Pulses**:

1. **Right Dorsalis Pedis**: +1 faint but detectable dorsalis pedis pulse
2. **Right Posterior Tibial**: +1 faint but detectable posterior tibial pulse
3. **Right Capillary Refill**: Normal CFT
4. **Right Vascular Skin Trophic Changes**: absent hair growth, decreased temperature gradient, pigmentation changes, and cool
5. **Right Edema**: Non pitting
6. **Right Venous Exam**: telangiectasia
7. **Sensation**: Right LE: Normal peripheral nerve sensation

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7. **Sensation Left LE:** Normal peripheral nerve sensation

- **Skin:**

Right Foot and Ankle: ulcer Wound bed granular less fat layer within the wound bed than previous. Does not probe deep. No fluctuance or purulence noted. No

other local signs of infection.

- **Toenails:**

Right: Normal

- **Inspection:**

1. Right Foot and Ankle: Charcot arthropathy
2. Appearance: well developed and nourished
3. Orientation: Alert and oriented to person, place, time.

- **Additional Exam Findings:**

1. Wound Width Pre-treatment in cm: 1.9
2. Wound Depth Pre-treatment in cm: .3
3. Wound Length Pre-treatment in cm: 2.9

- **Skin:**

Left Foot and Ankle: there is normal texture, temperature, turgor and color of the skin

- **Toenails Left:** Normal
- **Inspection:**

Left Foot and Ankle: Charcot Arthropathy

Wound Examination

Wound #	Location	Etiology	Stage	Length (cm)	Width (cm)	Depth (cm)	SA (cm^2)	Vol (cm^3)	Wound Bed
1	Left plantar foot (1st MTP)	DFU	Wagner 2	2.5	2.0	0.4	5.0	2.0	Pink/red
2	Left heel	DFU (pressure)	Wagner 1	3.0	2.5	0.2	7.5	1.5	Pink
3	Right medial ankle	Venous ulcer	Stage 3	4.5	3.0	0.3	13.5	4.0	Beefy red
4	Left lateral shin	Traumatic (laceration non-healing)	Stage 2	2.0	1.5	0.2	3.0	0.6	Red/pink

Wound #	Granulation %	Slough %	Eschar %	Exudate	Odor	Periwound	Undermining/Tunneling	Pain Palp.	Signs of Infection
1	60%	40%	0%	Moderate serosanguinous	Mild	Calloused	None	Y	Mild erythema

2	70%	30%	0%	Minimal clear	N	Macerated	None	Y	No infection
3	50%	50%	0%	Moderate serous	N	Hemosiderin staining, edema	None	N	No
4	80%	20%	0%	Mild serosanguinous	N	Intact	None	N	No

Assessment / Medical Necessity

- Patient has **4 chronic wounds**: 2 diabetic foot ulcers, 1 venous ulcer, 1 traumatic wound.
- Wounds have persisted **>8 weeks despite standard care (dressings, antibiotics, compression, off-loading attempts).**
- Contributing factors: Diabetes, peripheral neuropathy, venous stasis, PAD, poor compliance with compression/offloading, smoking.
- Risk factors if untreated: infection, hospitalization, limb loss, sepsis.
- **Medical Necessity:** Ongoing skilled wound care is required. Conservative management alone is insufficient. Advanced therapy (e.g., debridement, compression, CTP grafting) will be considered if inadequate progress continues.

Plan

Treatment	Details
Cleansing	NS irrigation for all wounds
Debridement	Selective sharp debridement of necrotic slough (Wounds 1, 3) – ~40% tissue removed
Dressing	Hydrofera Blue for DFUs, foam dressing for venous ulcer, bordered gauze for shin wound
Offloading/Compression	Total contact cast not tolerated; patient to use removable offloading boot. Multilayer compression for venous ulcer.
Nutritional guidance	Increase protein intake, multivitamins, diabetes diet adherence
Pain control	Acetaminophen + Gabapentin for neuropathic pain
Follow-up	Weekly reassessment, consider CTP after 4 weeks of documented SOC
Escalation plan	If no ≥50% wound area reduction by week 4 → initiate CTP grafts per LCD L35041
Grafting	Wound 1 & 2

Plan / Post-Procedure Care

- Dressings to remain intact x48 hrs unless saturated.

- Compression wraps to be replaced 2–3x/week.
- Offloading boot must be worn whenever ambulating.
- Monitor for infection (erythema, drainage, fever).
- Optimize comorbidities (tighten glycemic control, smoking cessation counseling).
- Follow-up in wound clinic in 7 days.

Diagnosis

Diagnosis	Code	Comments
Diabetic foot ulcer, left plantar, Wagner 2	E11.621 + L97.424	Non-healing, requires ongoing skilled care
Diabetic foot ulcer, left heel, Wagner 1	E11.621 + L97.421	
Venous leg ulcer, right medial ankle, Stage 3	I87.2 + L97.313	Chronic with stasis changes
Non-healing traumatic wound, left shin, Stage 2	T14.8XXA + L97.823	

Labs

- CBC
- HbA1c
- ESR/CRP if infection suspected

Follow-up

Type	Date	Notes
Office visit	09/30/2025	Weekly wound reassessment

Providers

- Referring Provider: Shaw, MD
- PCP: Karen White, MD
- Primary Provider: Bill Under, DPM

✅ **Compliance Checks:**

- Wound etiology, measurements, and characteristics documented for each wound
- Debridement type + % documented
- SOC elements documented (offloading, compression, infection control, vascular assessment, nutrition)
- Pain and comorbidities addressed
- Plan of care with escalation pathway documented
- Meets IL LCD L38902 & L35041 requirements